AiCare · Avenue Healthcare

Hospital membership, run like a health plan.

How Avenue can operate a provider-sponsored membership program with insurance-grade control, member-grade trust, and hospital-grade care flow.

MEMBER

CARE

FUND

AUDIT

Strategy presentation · 17 slides · Prepared for Avenue Healthcare

WHY THIS MATTERS NOW

Kenya's medical insurance market is leaking.

Avenue cannot operate a membership program on the same architecture that drained NHIF and pushed most insurers into years of losses.

KShs 33B

lost annually to healthcare insurance fraud in Kenya.

Association of Kenya Insurers

70%

of Nairobi medical insurers have been unprofitable for 5+ consecutive years.

Kenyatta University, AKI sector study

KShs 11B

blocked in fraudulent claims by SHA in just 6 months — controls work when they exist.

SHA reporting, Oct 2024 – Apr 2025

AVENUE'S STRATEGIC POSITION

Avenue is both the care provider and the fund steward.

That dual role is the opportunity — and exactly why generic insurance software does not fit. AiCare is built for the loop Avenue actually runs.

TRADITIONAL MODEL

Stitched-together stack

- External insurer
- Hospital information system
- Broker / TPA tools
- Separate claims processor
- Disconnected member portal

Five vendors. Five contracts. Five reconciliations.

AVENUE + AiCARE

One operating layer

- Membership & enrollment
- Pre-authorization & claims
- Provider contracts & tariffs
- Finance, billing & ledger
- Stakeholder portals

One platform. One source of truth. One audit trail.

HOW IT WORKS

One closed loop — from sale to care to settlement.

AiCare manages the full membership lifecycle as a connected flow — not as a series of departmental hand-offs.

01
Quote
Broker or admin builds the quote.

02
Enroll
Members & dependents added with checks.

03
Configure
Benefits, sub-limits, waiting periods set.

04
Verify
Member verified at the point of care.

05
Authorize
Pre-auth submitted, reviewed, approved.

06
Adjudicate
Claims checked against package & contract.

07
Settle
Provider payments and GL postings.

08
Learn
Signals feed pricing & renewals.

ROLES & PORTALS

Five workbenches, one platform.

Every actor in the program — from the head of finance to a member checking their benefits — gets a workbench built for the work they actually do.

ADMIN	HR MANAGER	MEMBER	BROKER	FUND ADMIN
Runs the program.	*Manages the roster.*	*Owns their care.*	*Sells and renews.*	*Stewards the pool.*
Members	Roster	Member card	Submissions	Deposits
Claims	Endorsements	Benefits	Quotations	Live balance
Packages	Invoices	Dependents	Commissions	Deductions
Providers	Utilization	Pre-auth status	Renewals	Holds
Fraud & audit	Service requests	Account security	Client support	Statements

PRODUCT MODEL

Configurable membership products.

One engine — corporate, group, or individual. Every value editable per package version, without rebuilding.

THE BUILDING BLOCKS

1. **Packages & versions**
   Each plan has dated versions for clean history.
2. **Benefit categories**
   Outpatient, inpatient, maternity, dental, optical, etc.
3. **Sub-limits**
   Annual or per-event caps per category.
4. **Waiting periods**
   Per benefit, with overrides for groups.
5. **Co-contributions**
   Percentage or flat-rate member share.

EXAMPLE: AVENUE EXECUTIVE PACKAGE

Benefit	Annual sub-limit	Waiting	Co-pay
Outpatient	KShs 250,000	30 days	10%
Inpatient	KShs 1,000,000	90 days	0%
Maternity	KShs 150,000	12 months	15%
Dental	KShs 40,000	60 days	20%
Optical	KShs 30,000	60 days	20%

Example only. Every value is editable per package version. The same engine drives corporate, group, and individual products.

MEMBERSHIP CONTROL

Enrollment becomes a controlled event.

Most programs quietly accumulate errors at this stage. AiCare turns every member change into an auditable event with billing impact calculated up-front.

1 HR REQUEST

- Add dependent
- Remove member
- Transfer to a new group
- Update profile or category

2 ADMIN REVIEW

- Validate eligibility
- Set effective date
- Confirm package version
- Preview billing impact

3 RESULT

- Roster updated
- Invoice adjusted
- Benefit state changed
- Audit trail recorded

Member changes are events, not silent admin edits — and the billing impact is visible before anyone clicks approve.

POINT-OF-CARE VERIFICATION

Check-in is a security event, not a card swipe.

Identity is confirmed by a biometric prompt on the member's own phone, plus a matching code at the reception desk — the digital equivalent of two people opening a vault together.

STEP 01

Reception scans

Member identified at the desk by QR or queue.

STEP 02

Member confirms

Face or fingerprint tap on the member's phone.

STEP 03

Codes match

A short code on both screens — they must match.

STEP 04

Visit opens

Signed, time-stamped, entered into the audit chain.

Kills the most common Kenyan medical insurance fraud · *'servicing non-members' — at the front door.*

Emergency override available, reviewed within 24 hours.

PRE-AUTH & CLAIMS

The clinical-financial spine.

Pre-auth and claims run on the same engine. Every decision is evaluated against the full membership context — not just what's on the invoice.

A SINGLE CLAIM SEES…

Member status	*Active, on package version XXXX*
Check-in verified	*Biometric confirmed at reception*
Provider contract	*Tariff and rates matched*
Service lines	*ICD-10 + CPT validated*
Sub-limit balance	*Outpatient cap not exceeded*
Co-contribution	*Member share calculated up-front*
Fund balance	*Self-funded pool sufficient*
Fraud signals	*No anomaly pattern detected*

WORKBENCH

What the user works with

- Service lines by category
- ICD-10 + CPT search
- Tariff variance flags
- Document attachments
- Exception routing
- Co-pay collection

Pre-auth uses the same engine — same checks, just before service rather than after.

FRAUD CONTROL

Fraud control is embedded — not bolted on at the end.

A provider-sponsored program has a structural risk: the hospital is also the insurer. AiCare scrutinizes Avenue's own claims with the same rigor it applies to external referrals.

LAYER 01

Gate checks

Identity, eligibility, and payment verified before service begins.

LAYER 02

Rules engine

Tariff variance, frequency limits, gender and age sanity, pathway checks.

LAYER 03

Anomaly detection

Provider, member, broker, and check-in patterns watched continuously.

LAYER 04

Audit chain

Every override, flag, and investigation outcome permanently retained.

NHIF Linda Mama: 656 caesarean sections billed for one patient — KShs 5.7M. **AiCare assumes no actor — internal or external — is automatically low-risk.**

FINANCE & ACCOUNTING

Insurance-grade discipline. Membership-grade language.

On the surface, AiCare uses the language Avenue is allowed to use under Kenyan regulation. Underneath, it runs the discipline an insurance pool actually requires.

ON THE SURFACE

Membership language

USES	DOES NOT USE
Member	*Premium*
Contribution	*Policy*
Package	*Cover*
Benefit	*Deductible*

UNDER THE HOOD

Insurance-grade finance

- Billing runs
- Invoices & dunning
- Payment matching
- Ledger entries
- GL posting
- Audit reports

Contribution › **Invoice** › **Payment** › **Claim** › **GL post** › **Report**

Each step generates accounting entries — a real ledger, not a spreadsheet.

SELF-FUNDED SCHEMES

Self-funded pools, in the open.

When a corporate client deposits a fund, AiCare gives them a live view of where their money is. The hospital becomes a transparent administrator — not a black box that emails PDFs once a quarter.

ABCD LTD · Self-funded pool LIVE

KSHS AVAILABLE

4,287,000

68% of starting balance remains

Last deposit	KShs 2,000,000	*12 Apr 2026*
Claims paid	KShs 1,648,000	*Apr to date*
Admin fee	KShs 65,000	*Apr accrued*

FUND CONTROL SURFACE

- **Deposits & top-ups**
  Logged with date and reference.
- **Live balance view**
  Updated every claim approval.
- **Claim deductions**
  Auto-applied per benefit category.
- **Admin fee billing**
  Flat or % of claims.
- **Category holds**
  Cap a benefit when limit hit.
- **Statements & exports**
  PDF, Excel, accounting integration.

Employers running self-funded schemes are the audience that compares vendors most carefully — this surface wins those conversations.

MEMBER EXPERIENCE

More than an eligibility check.

When members can self-serve their benefits, dependents, pre-auth status, and security, support load drops and trust goes up. The portal does the work HR and customer service used to do.

Lives in the member's wallet. No plastic. No re-issues.

Digital member card
QR + passkey, no plastic.

Dependents
Family members and relationships.

Benefits & utilization
Limits used, limits remaining.

Pre-auth status
Live updates, no calls.

Facility finder
Network search and directions.

Account security
Member-managed device list.

Every question a member can answer themselves is a call HR and the service desk don't have to take.

HEAD TO HEAD

Where AiCare differs from the alternatives.

Most competitors solve a single department's problem and patch the rest. AiCare was designed for the dual role Avenue actually plays — provider and fund steward.

CATEGORY	TYPICAL COMPETITOR	AiCARE
Verification	*Plastic card or SMS OTP at the desk.*	**Passkey + dual-code, with audited override.**
Architecture	*Separate insurer, hospital, broker, and TPA tools.*	**One hospital-owned operating layer.**
Fraud control	*Reviewed after service. Hard to recover.*	**Checked at enrollment, pre-auth, visit, claim, and payment.**
Self-funded	*Quarterly statements emailed as PDF.*	**Live fund ledger with low-balance alerts and category holds.**
Portals	*Status viewers — read-only at best.*	**Workbenches where actors do their real work.**

STAKEHOLDER VALUE

What each side actually gets.

A platform only succeeds when every actor in it would prefer it to the alternative. Here is what each one gets.

HOSPITAL

Runs and underwrites the program.

- Embedded fraud control
- Utilization insight
- Faster settlement
- One operating layer

EMPLOYER

Pays for and oversees coverage.

- Audited roster control
- Invoice clarity
- Utilization reports
- Live fund view

MEMBER

Uses the program for their family.

- Visible benefits
- Digital member card
- Biometric check-in
- Self-service portal

BROKER

Sells, renews, and supports.

- Quotations
- Group submissions
- Commission tracking
- Renewals pipeline

ADOPTION

A practical path, not a big-bang launch.

Each phase makes the next one easier and reduces the risk of any single misstep. Avenue chooses the pace, but the sequence holds.

01

PHASE 01

Stabilize core

- Packages & groups
- Memberships
- Billing & claims
- Standard reports

PROVE THE OPERATING LOOP

02

PHASE 02

Activate controls

- Secure check-in
- Tariff variance
- Fraud desk
- Audit reviews

CLOSE THE FRAUD DOORS

03

PHASE 03

Open portals

- HR self-service
- Member portal
- Broker workbench
- Fund admin view

REDUCE SUPPORT LOAD

04

PHASE 04

Optimize pool

- Utilization analytics
- Renewal pipeline
- Pricing feedback
- Service trends

TURN DATA INTO MARGIN

Avenue chooses the pace. Each phase de-risks the next and proves value before scope expands.

WHAT NEXT

Simple at the front door.
Rigorous behind the scenes.

Avenue can offer a membership program that members trust, brokers can sell, employers can audit, and the hospital can defend — built on one platform, not a stitched-together stack.

BOOK A 60-MINUTE WORKING SESSION

Bring clinical, finance, and IT leads. We will demo live on Avenue's data shape.

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